

The procedure Since it's important for you to remain still during the procedure, the anesthesiologist will insert the epidural between your contractions. If this is difficult, you should try to concentrate on your breathing and remain as still as possible. You will feel a pushing sensation in your back while the anesthesiologist is trying to find the very small epidural space with the hollow needle. When the space is located, a tiny plastic tube will be fed into it through the needle. The epidural needle is then removed and the tube, which is secured onto your back with sticky tape, remains in the epidural space. The tube remains in place until your baby is delivered and, because it is very thin, soft, and pliable, it is perfectly safe to lie on the tube and to move around.

Managing the epidural Once the epidural tube is successfully in place, the anesthesiologist will give the first dose of medication. Once they are satisfied that the epidural is in the correct position and is working effectively, all subsequent doses, or "top offs," can be given without another injection. Your blood pressure will be taken once the epidural is in place and will be monitored for the next half an hour or so, and then regularly thereafter, including after each top off. Each dose of medication takes around 10–20 minutes to take its full effect and can last between one and two hours. The epidural will be topped off as required, usually around every three to four hours, to keep you comfortable throughout your labor. An anesthesiologist should be available 24 hours a day to manage any concerns or problems that may arise with the epidural.



Before the epidural is given, your back will be covered with a sterile sheet and then a local anesthetic will be given to numb the area so that you don't experience pain when the larger epidural needle is inserted.